

FAX

**The University of Texas
MD Anderson Cancer Center**

DATE: 12/11/24

TO: Tricia Fredericks, MD

FROM: Bercow, Alexandra, MD
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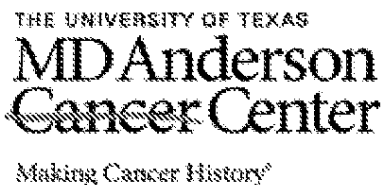
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*A Comprehensive Cancer Center designated by the National Cancer Institute
located in the Texas Medical Center*



Patient: Elizabeth Ann Martin
Date of Birth: 9/1/1955
Sex: Female
MRN: 2881197

December 11, 2024

Tricia Fredericks, MD
800 Rose St
Lexington KY 40536

Dear Colleague,

I have recently seen our mutual patient, Elizabeth Ann Martin, at The University of Texas MD Anderson Cancer Center. We value our relationship with you and appreciate your confidence in our service and expertise. It is our goal to provide this patient with the highest quality of care in the most efficient manner.

Attached you will find documentation from my recent encounter with this patient. You may also obtain access to patient documentation and test results any time by logging into myMD Anderson physician portal, powered by EpicCare Link. If you are not currently registered, you may do so by following this link:

<https://mylink.mdanderson.org>

If you have questions or concerns regarding your patient, please contact me as indicated below. You may also choose to send an In Basket message through EpicCare. We look forward to working with you in providing the best possible care for Elizabeth Ann Martin.

Regards,

Alexandra Bercow, MD
Office Phone Number: 713-792-6810
Gynecologic Oncology Center
The University of Texas MD Anderson Cancer Center

Enclosure

Johnson, Cynae Alonia Lillian, APRN 12/11/2024 3:28 PM Attested

Reason for Visit: Second Opinion for recurrent uterine serous carcinoma

Subjective

History of Present Illness:

Elizabeth Ann Martin is a 69 y.o. female with a history of stage IIIB uterine serous carcinoma.

She was initially diagnosed in 11/2023 when an EMB revealed high grade endometrial adenocarcinoma, favor serous. Pre-operative CT was negative for metastatic disease or lymphadenopathy. She underwent RA-TLH, BSO SLNBx, and biopsy of sigmoid colon nodule on 12/14/23 by Dr. Hope Cottrill at Baptist Healthcare System. Pathology revealed serous carcinoma involving primarily an endometrial polyp with myometrial invasion (DOI 8/15mm, 53%, tumor size 2.5 x 2.5 x 1.9 cm, No LVSI), left fallopian tube, sigmoid colon, and +washings. She transferred care to Dr. Tricia Fredericks at University of Kentucky and initiated adjuvant treatment with Carboplatin/Taxol. Patient had a HSR to Taxol with C2, and was able to tolerate subsequent cycles with rate reduction and additional premedication for a total of 6 cycles, last on 5/17/24. Per patient report, interval imaging was in 7/2024 was negative for disease. She continued with adjuvant brachytherapy x3 Fx, completed 8/14/24.

She returned to clinic for routine surveillance visit on 12/4/24. PETCT on 11/29/24 revealed FDG avidity along the left lateral aspect of the vaginal cuff, and peritoneal thickening along the left pelvic sidewall and nodularity in the LUQ concerning for metastatic disease. She was recommended to restart chemotherapy with Doxil/Avastin vs Pembrolizumab/Lenvatinib. She presents today for a second opinion.

CARIS testing revealed (MSS, LOH 9%, TMB 3, ER +, HER2 neg, PR +, PDL1 neg, HLA 23:01, 29:02). Germline testing with Ambry Genetics (6/10/2024) was + for SDHA (5'UTR_EX9del), and AXIN2 variant of unknown significance (c.2141+4A>G).

Cancer Staging

Malignant neoplasm of uterus

Staging form: Corpus Uteri, FIGO 2015

- Clinical stage from 12/14/2023: Stage IIIB (Recurrent) - Unsigned

Oncology History

Malignant neoplasm of uterus

	Initial Diagnosis
	Malignant neoplasm of uterus
11/2023	DX-Biopsy EMB - high grade endometrial adenocarcinoma, favor uterine serous
12/14/2023	TX-Surgery RA-TLH, BSO, SLN, biopsy of sigmoid colon. Pathology stage IIIB, serous carcinoma involving primarily an endometrial polyp with myometrial invasion (DOI 8/15mm, 53%, tumor size 2.5 x 2.5 x 1.9 cm, No LVSI), left fallopian tube, sigmoid colon, and +washings Ca 125 16.2

Patient: Elizabeth Ann Martin

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1/19/2024 - 5/17/2024	TX-Chemotherapy/Biotherapy/Investigational/SMI Carboplatin/Taxol x6 - HSR w/ C2 able to proceed with reduced rate
7/25/2024 -	Status-No Evidence of Disease (NED) Based on CT findings
7/30/2024 - 8/14/2024	TX-Radiation Therapy Vaginal brachytherapy x3, 7/30, 8/7, 8/14
11/29/2024 -	Status-Relapse/Recurrence PETCT: Hypermetabolic activity along the left lateral aspect of the vaginal cuff concerning for local recurrence. Hypermetabolic peritoneal thickening along the left pelvic sidewall and nodularity in the left upper quadrant concerning for metastatic disease. CA 125 94.8

Review of Systems

Constitutional: Negative for appetite change, chills, diaphoresis, fatigue, fever and unplanned weight loss.

HENT: Negative for ear pain, mouth sores, nosebleeds, rhinorrhea, sore throat, trouble swallowing and voice change.

Breast: Negative for adenopathy.

Skin: Negative for alopecia, itching and rash.

Respiratory: Negative for cough, shortness of breath and wheezing.

Cardiovascular: Negative for chest pain, palpitations and leg swelling.

Gastrointestinal: Positive for bloating and constipation. Negative for blood in stool, diarrhea, nausea and vomiting.

Endocrine: Negative for cold intolerance, heat intolerance, hot flashes, polydipsia and polyuria.

Genitourinary: Positive for bladder incontinence and pelvic pain (LLQ, intermittent takes Tylenol prn). Negative for decreased urine volume, difficulty urinating, hematuria and vaginal discharge.

Musculoskeletal: Positive for arthralgias (left knee, sees OSI ortho). Negative for back pain, joint swelling, neck pain and neck stiffness.

Eyes: Negative for visual disturbance.

Hematological/Lymphatic: Does not bruise/bleed easily.

ECOG Performance Status:

1 - Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g., light house work, office work

Allergies

Allergen	Reactions
• Paclitaxel	Anxiety, Other (See Comments) and

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Encounter Date: 12/11/2024

Shortness Of Breath

Shortness of breath, back pain, flushing, headache.

Current Medications

Medication	Sig	Start Date	End Date	Taking?	Authorizing Provider
acetaminophen (TYLENOL) 325 mg tablet	Take 2 tablets (650 mg) by mouth.	12/14/23		Yes	Provider, Historical, MD
amlodipine (NORVASC) 5 mg tablet	TAKE 1 TABLET ORALLY ONCE PER DAY FOR 30 DAYS			Yes	Provider, Historical, MD
citalopram (CeleXA) 10 mg tablet	TAKE 1 TABLET ORALLY ONCE PER DAY FOR 30 DAYS			Yes	Provider, Historical, MD
diphenhydramine (BENADRYL) 25 mg capsule	Take 1 capsule (25 mg) by mouth.			Yes	Provider, Historical, MD
lisinopril (PRINIVIL, ZESTRIL) 10 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY			Yes	Provider, Historical, MD
LORazepam (ATIVAN) 1 mg tablet		3/27/23		Yes	Provider, Historical, MD
tramadol (ULTRAM) 50 mg tablet	Take 1 tablet (50 mg) by mouth.	3/29/24		Yes	Provider, Historical, MD

OB History

Gravida	Para	Term	Preterm	AB	Living
2	2				
SAB	IAB	Ectopic	Multiple		Live Births

#	Outcome	Date	GA	Lbr Len/2nd	Weight	Sex	Type	Anes	PT	Lv
2	Para									
1	Para									

Past Medical History:

Diagnosis	Date
• Anxiety <i>Controllable mild anxiety for several years, but some worse the past year.</i>	2011
• Arthritis <i>Knee pain</i>	2018
• Hypertension <i>on meds for at least 10 years</i>	2014

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- Malignant melanoma
completely excised. January 2016
- Malignant neoplasm of uterus November 2023
- *Endometrial Serous Carcinoma*
- Menopause 2010
- Neuropathy Jan 2024
started with chemo

Past Surgical History:

Procedure	Laterality	Date
• COLONOSCOPY <i>2nd one-March 2021</i>		March 2016
• HYSTERECTOMY		December 2023

Family History

Problem	Relation	Age of Onset
• Skin cancer	Mother	
• Endometrial cancer <i>When she was 68 years old.</i>	Mother	
• Hypertrophic cardiomyopathy	Mother	
• Kidney cancer	Father	
• Kidney cancer	Sister	
• Skin cancer	Brother	

Social History**Socioeconomic History**

- Marital status: Married
- Spouse name: Not on file
- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Never
- Smokeless tobacco: Never

Substance and Sexual Activity

- Alcohol use: Never
- Drug use: Never
- Sexual activity: Not Currently
- Partners: Male
- Birth control/protection: Post-menopausal

Other Topics

- Not on file

Social History Narrative

Patient: Elizabeth Ann Martin

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- Not on file

Objective

Wt 81.6 kg (179 lb 14.3 oz) | Ht 156 cm (5' 1.42") | BP 150/79 | Pulse 88 | Temp 37 °C (98.6 °F) (Oral) | Resp 16 | SpO2 96%

Physical Exam

Vitals and nursing note reviewed.

Constitutional:

Appearance: She appears well-developed and well-nourished.

HENT:

Head: Normocephalic and atraumatic.

Eyes:

Extraocular Movements: EOM normal.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds.

Abdominal:

General: Bowel sounds are normal. There is no distension.

Palpations: Abdomen is soft. There is no mass.

Tenderness: There is no abdominal tenderness.

Genitourinary:

Visual Exam: External female genitalia normal and urethral meatus normal. Vulva exhibits no vulval lesions.

Speculum Exam: Vagina normal. Vagina exhibits no bleeding, no lesions and vaginal discharge.

Bimanual Exam: There is no palpable mass and no palpable nodularity. Adnexa: no fullness, tenderness, or masses appreciated. Cervix and Uterus surgically absent

Rectovaginal Exam: Normal. There is no palpable mass and no palpable nodularity.

Musculoskeletal:

General: Normal range of motion.

Cervical back: Normal range of motion.

Lymphadenopathy:

No other adenopathy present.

Skin:

General: Skin is warm and dry.

Findings: No erythema.

Neurological:

Mental Status: She is alert and oriented to person, place, and time.

Psychiatric:

Mood and Affect: Mood and affect normal.

Assessment & Plan

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Encounter Date: 12/11/2024

Elizabeth Ann Martin is a 69 y.o. female with a history of stage IIIB uterine serous carcinoma.

1. Malignant neoplasm of endometrium (Primary)

- We reviewed the natural history and progression of uterine serous carcinoma and discussed the incurable nature of her disease due to recurrence. We discussed treatment options which include standard of care chemotherapy vs clinical trials. The patient would like to proceed with SOC treatment at this time.
 - A treatment recommendation sheet for Pembrolizumab/Lenvatinib was provided. We briefly reviewed the side effects with the patient. Patient is recommended to receive 3 cycles followed by repeat imaging and treatment planning visit at MD Anderson.
 - Baseline labs collected today. We will follow up on OSI pathology review from her hysterectomy. We will store her PETCT from 11/29/24 for future comparison.
 - Plan to also obtain actual CARIS testing report for review.
 - All questions/concerns addressed.
-
- Hepatitis C Virus Antibody; Future
 - Hepatitis B Surface Ag; Future
 - HIV 1/2 Antigen/Antibody, Fourth Gen W/RFL; Future
 - Hemoglobin A1c; Future
 - Glucose, Random; Future
 - BUN; Future
 - Creatinine; Future
 - Fractionated Bilirubin; Future
 - ALT; Future
 - Aspartate Aminotransferase; Future
 - Electrolyte Panel; Future
 - Magnesium Level; Future
 - Complete Blood Count with Differential; Future
 - Albumin Level; Future
 - CA 125; Future

This was a shared visit. I am not the billing provider but have participated in the care of this patient and agree with the visit notes. I spent 45 minutes of reportable time on the date of the visit.

Follow up plan: interval treatment planning following initiation of chemotherapy.

Attestation signed by Bercow, Alexandra, MD at 12/11/2024 3:28 PM:
Gynecologic Oncology Co-managed Addendum:

I have seen the patient today with Cynae Johnson APRN, and this is a co-managed addendum. I agree with the history, review of systems, physical examination, and assessment and plan as outlined in her note.

I have personally reviewed the patient's vital signs, medications, allergies, past medical history, laboratory blood work, associated radiologic studies, and I personally performed a physical examination.

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Briefly, she is a 69 y.o. with pMMR, p53 mutant, HER2 0+ (by IHC) stage IIIB uterine serous carcinoma s/p TRHBSO, LSLND, and resection of bowel serosal implants to R0 f/b 6C adjuvant carbo/taxol (completed 5/17/2024) and VBT (completed 8/2024) who presents after diagnosis of a likely recurrence at the vaginal cuff and left sidewall. She is currently asymptomatic aside from minor LLQ discomfort occasionally. Denies vaginal bleeding, abnormal discharge, changes in bowel habits, anorexia, nausea, fevers, abdominal/pelvic pain. Her gynecologic oncologist at UK (Dr. Fredericks) recommended she start doxil/bev as she felt the side effect profile of lenvatinib/pembrolizumab may be prohibitive. She presents today for a second opinion. Of note, per report, CARIS testing demonstrated mutations in PPP2R1A and FGFR2.

ECOG Performance Status: 1 - Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g., light house work, office work

PHYSICAL EXAMINATION:

VITAL SIGNS: BP 150/79 | Pulse 88 | Temp 37 °C (98.6 °F) (Oral) | Resp 16 | Ht 156 cm (5' 1.42") | Wt 81.6 kg (179 lb 14.3 oz) | SpO2 96% | BMI 33.53 kg/m²

GENERAL: The patient is awake, alert and in no acute distress.

HEENT: Normocephalic with facial symmetry, no scleral icterus.

NECK: Supple. No thyromegaly.

LYMPH NODES: There is no palpable cervical or supraclavicular lymphadenopathy.

ABDOMEN: Soft, nondistended, nontender. No palpable masses. No rebound or guarding.

PELVIC: normal external genitalia and vaginal cuff without lesions. Uterus/adnexa surgically absent. Rectovaginal exam without nodularity.

EXTREMITIES: warm and nontender, without edema.

NEUROLOGIC: Grossly normal.

SKIN: No rashes

In summary, Elizabeth Ann Martin is a 69 y.o. female with recurrent pMMR, p53 mutant, HER2 0+ (by IHC) uterine serous carcinoma s/p surgery (12/2023), adjuvant carbo/tax (completed 5/2024), and VBT (completed 8/2024) who presents with a small volume pelvic recurrence.

We reviewed the literature and evidence surrounding recurrent uterine serous carcinoma as well as the tumor genetic mutations. KEYNOTE-775 demonstrated a PFS and OS benefit from lenvatinib/pembrolizumab compared to physicians choice chemotherapy in advanced, recurrent or metastatic endometrial cancer. The pMMR subgroup population in the lenvatinib/pembrolizumab arm still experienced a PFS and OS benefit compared to those who received systemic chemotherapy. As such, this patient would likely benefit from lenvatinib/pembrolizumab compared to a systemic chemo like doxil/bev and I would reserve doxil/bev as an option for later lines given how well it is tolerated. The patient has an excellent performance status and while she may need a dose reduction, should tolerate lenvatinib/pembrolizumab well. We also discussed that her cancer has several targetable mutations (PPP2R1A and FGFR2) that may qualify her for some of our Phase I clinical trials but at this time she is not interested in exploring those further.

I have reviewed her active issues:

Problem List Items Addressed This Visit

Genitourinary

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Malignant neoplasm of uterus - Primary

Relevant Orders

Hepatitis C Virus Antibody
Hepatitis B Surface Ag
HIV 1/2 Antigen/Antibody, Fourth Gen W/RFL
Hemoglobin A1c
Glucose, Random
BUN
Creatinine
Fractionated Bilirubin
ALT
Aspartate Aminotransferase
Electrolyte Panel
Magnesium Level
Complete Blood Count with Differential (Completed)
Albumin Level
CA 125

Her plan is as follows:

- Recommended she start with lenvatinib 20mg daily and pembrolizumab 200mg q3 weeks for 3 cycles and then return for further imaging and treatment planning. She will discuss with her primary oncologist and see if she can receive it under her direction at UK. If not, she will return here for treatment.
- Will follow up her baseline labs today
- Will ensure we receive her path fr review as well as her original CARIS report

This was a shared visit and I performed the substantive portion of the visit based on spending the majority of time. The level of service was chosen based on time. This includes time spent with the patient and/or patient's family, and time preparing for the visit, completing documentation, and coordinating care. The total time I spent on the date of the visit was 60 minutes. None of our time was overlapping. The combined total time on the date of the visit was 90 minutes.

Alexandra Bercow, MD
Assistant Professor
Gynecologic Oncology and Reproductive Medicine
University of Texas MD Anderson Cancer Center
C 823-628-6771